



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نجلاء عطا محمد احمد  
المحافظة : القاهرة الحي / المدينة : المطرية  
رقم البطاقة : 28602100103601 رقم التليفون : 01159645688  
العنوان : \*  
العمر (تاريخ الميلاد) : 10-02-1986 المهنة : ربه منزل  
تاريخ تسجيل الإستمارة : 24-09-2026

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (-----) المقرر بما فيه  
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني  
الاسم : د/ندى حسن  
الاسم : د/نرمين رمضان  
الوظيفة :  
الوظيفة :  
الوظيفة :  
التوقيع :  
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التوقيع :  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.  
تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. / د. / د. / د. / د.

### قرار اللجنة

التشخيص النهائي :  
خطة العلاج :  
المستشفى :  
جهة العلاج :  
ملاحظات :  
قسم :  
نفقات :  
ملاحظات :

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
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|  |
|--|

|                                                                        |                                                                                     |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> نجلاء عطا محمد احمد<br><b>National ID:</b> 28602100103601 |  |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

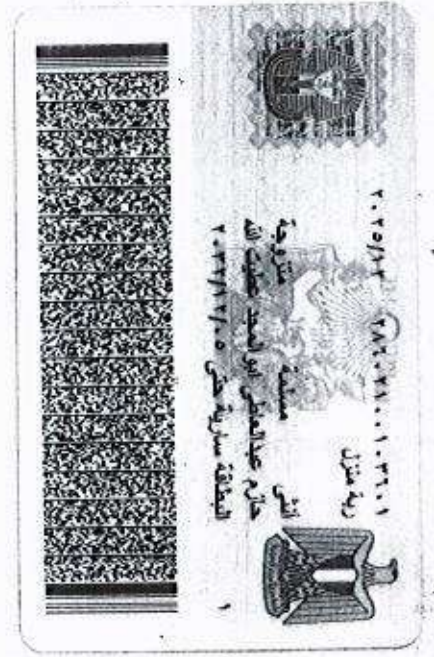
|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                            |
|--------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound |
|--------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



Handwritten signature in blue ink, appearing to read 'DMS' followed by a stylized signature.

04/12/2026

## MDT FORM

|                                         |                                |
|-----------------------------------------|--------------------------------|
| <b>Name:</b> تولا، عطا محمد احمد        | <b>MRN:</b> 9096               |
| <b>Age:</b> 40 Y                        | <b>ID:</b> 28602100103601      |
| <b>Menopausal Status:</b> PREMENOPAUSAL | <b>Phone:</b> 01122895165      |
| <b>NRS SCORE:</b>                       | <b>Marital Status:</b> MARRIED |
| <b>BMI:</b>                             | <b>Family History:</b>         |
| <b>CO Morbidity:</b> NOT dm OR htn      | <b>Reason Of MDT:</b>          |

## Clinical Data:

**BX:** IDC G2

**ER** 0/8 **PR** 0/8 **HER** +3 **KI67**

## Sonomammography:

## Mets Workup:

## History:

A case of female pt with 39 yrs , not DM , HTN , No major operation , Premenopausal , Had 2 offspring , +ve uncle stomach

pt underwent BCS + AC --> T2 (2x2cm) N7/13

ER 0/8 , PR 0/8 , Her2 +ve

pt received 4 AC + 12 wks Taxol + 2 phasgo in private

PET CT --> free

CT CAP --> free

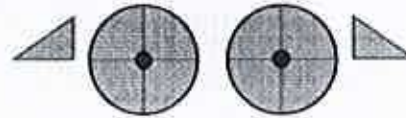
## Recommendations:

## MDT Outcome & Plan

continue Phasgo + PORT

R

L



T N M

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|------------------|---------|-----------|-----------|-----------|-------------|
| د/ نرمين مصطفى   |         |           |           |           |             |

|                                         |                                                |
|-----------------------------------------|------------------------------------------------|
| Patient Name : الأستاذ / نجلاء عطا محمد | Request No. : 13005187                         |
| Patient No. : 28602100103601            | Request Date : 24/02/2026                      |
| Path No. : SR-299/26                    | Verification Date : 28/02/2026                 |
| Sex : Female                            | Financial Category : جوستاف روسي - نفقة الدولة |
| Age : 40 Y                              | Department : Out Patient                       |
| Referring Doc :                         | Registered by : Mrs.Mayada Hegazy              |

### Pathology Report

Clinical Data: Right breast cancer.

Nature of Specimen: Referred block & slides (Right breast conservative breast surgery with axillary clearance).

#### Gross Examination:

- One referred block labelled by (5972-25-1). Together with, thirteen referred slides including 5 (H&E) stained slides & 8 immunostained slides for (Two for ER, Two for PR, Two for Her2, One for SISH & one for Ki67).

#### Microscopic Examination:

- \* Histological type: Extensive autolytic changes, Invasive duct carcinoma.
- \* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- \* DCIS: Focal intermediate grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).
- \* Angioinvasion: Present.
- \* Perineural invasion: Absent.
- \* Skin: Free.
- \* Surgical margins: Refer to the original report.
- \* Axillary lymph nodes: Positive metastatic deposits in seven out of thirteen lymph nodes (7/13) with evidence of extra-nodal extension.
- \* IPT:
- ER (Both referred slides): Negative nuclear reaction in tumor cells (score 0/8).
- PR (Both referred slides): Negative nuclear reaction in tumor cells (score 0/8).
- Her2 (Both referred slides): Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH (One slide): Amplified with small clusters.
- Ki67% (One slide): Unreliable.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذة / نجلاء عطا محمد  
Patient No. : 28602100103601  
Path No. : SR-299/26  
Sex : Female  
Age : 40 Y  
Referring Doc :

Request No. : 13005187  
Request Date : 24/02/2026  
Verification Date : 28/02/2026  
Financial Category : جوستاف روسي - نفقة الدولة  
Department : Out Patient  
Registered by : Mrs.Mayada Hegazy

### Pathology Report

#### Diagnosis:

Right breast cancer, Referred block & slides (Right breast conservative breast surgery with axillary clearance):

- Extensively autolytic specimen, Invasive duct carcinoma, Grade 2.
- Focal intermediate grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).
- Skin: Free.
- Axillary lymph nodes: Positive metastatic deposits in seven out of thirteen lymph nodes (7/13) with evidence of extra-nodal extension.
- \* Stage: pN2a.

#### IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with small clusters.

\* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

\* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0+ (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

Verified by :

Dr.Duaa Mohamed

Printing Date : 08/04/2026 12:12:56

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Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

|                                          |                                                |
|------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذة / نجلاء عطا محمد | Request No. : 13005187                         |
| Patient No. : 28602100103601             | Request Date : 24/02/2026                      |
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| Age : 40 Y                               | Department : Out Patient                       |
| Referring Doc :                          | Registered by : Mrs.Mayada Hegazy              |

### Pathology Report

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

Signature  
Dr. Walaa Ibrahim Ghoneim.

Verified by :  
Dr.Duaa Mohamed



PATIENT NAME: NAGLAA ATAH MOHAMMED AHMED

GENDER: Female

DATE : 28-Oct-2025 10:35 AM

PATIENT ID: 10125271588

AGE: 39 y

ORDER: 3603414

Whole body [18F] FDG PET-CT ScanCLINICAL HISTORY:

-39years old patient, presented with managed right breast cancer, underwent right CBS and received chemotherapy.

Reason for the study: detection of FDG avid neoplastic lesions.

TECHNIQUE:

- Radiopharmaceutical: 18F FDG, intravenously.
- Time from 18F-FDG injection to scan: 60 min
- PET/CT images were acquired from the skull vault through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and Sagittal planes.
- Patient's weight: 85 Kg - Patient's height: 156cm -Bl. Gluc. Level: 133 mg/dl
- Additional notes: SUV max normalized to body weight was used.
- Reference hepatic activity (SUV max): -3.2

FINDINGS:Breast and axillary regions:

- Status post right CBS with no FDG avid residual /recurrent lesions. Associated with low grade mild diffuse skin thickening.
- Low grade right axillary ill defined soft tissue density, likely post therapeutic changes. No FDG avid axillary lymphadenopathy.
- No FDG avid left breast lesions or axillary lymphadenopathy.

Head and Neck:

- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands.
- No FDG avid focal thyroid lesions.
- No hypermetabolic or pathologically enlarged cervical lymphadenopathy.

PATIENT: NAGLAA ATAH MOHAMMED AHMED, 10125271588, Female, 39 y

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الشارع الرئيسي - شارع البيرة بجوار دارى الحيرة

الشارع الرئيسي - شارع البيرة بجوار دارى الحيرة

PET Scan

[[DOCTYPE:PET\_SCAN]]

#### Chest:

- No FDG avid mediastinal lymph nodes.
- No FDG avid lung nodules or masses detected.
- Right middle lobe fibrotic band, post radiation effect.
- No metabolically active pericardial or pleural lesions detected.

#### Abdomen and Pelvis:

- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected..
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas and both supra-renal with no FDG avid lesions detected.
- No evidence of ascites or abdominal collections noted.
- No FDG avid abdominal or pelvic nodal lesions.
- Bulky uterus with lobulated outline with no FDG avid focal lesions, for US assessment.

#### Musculoskeletal:

- Normal FDG activity is seen over the axial skeleton.
- No focal active blastic or lytic bone lesions detected.
- Non-FDG avid left humeral head small sclerotic osseous focus.
- Bilateral sacro-illitis; more at the left.

#### Conclusion:

**FDG PET/CT study for a known case of managed left breast cancer revealed;**

- Evidence of right CBS with no FDG avid residual /recurrent neoplastic lesions.
- No FDG avid loco-regional or distant metastatic lesions.

Reported By,

Dr. Alaa ElSaeed

Dr. Mahmoud Ibrahim, MD

Ass. Prof. Mai Amr, Nuclear Medicine Consultant

29-Oct-2025 02:48 PM

PATIENT: NAGLAA ATAH MOHAMMED AHMED, 10125271588, Female, 39 y

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4 شارع بلوية اول شان الهرم بجوار داي الحيرة

د. التي الاول - الشيخ زيد امام كلية الهندسة

PET Scan

[[DOCTYPE:PET\_SCAN]]

PATIENT NAME: NAGLAA ATAH MOHAMMED AHMED

GENDER: Female

DATE : 06-Apr-2026 03:55 PM

PATIENT ID: 10125271588

AGE: 40 y

ORDER: 3817548,3817834

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,  
ABDOMEN AND PELVIS WITH TRIPHASIC LIVER STUDY****Morphological correlation with previous PET CT dated 28/10/2025****FINDINGS:****1-CHEST**

- Status post right BCS showing operative bed distortion and skin thickening with no sizable gross recurrent lesions.
- Middle lobe and lingula atelectatic bands.
- Left lower lobe posterior segment pleural tags.
- Stationary few bilateral milimetric pulmonary nodules ranging from 1 to 2 mm likely non specific.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Left sided porta cath.
- Thyroid isthmus hypodense nodule measuring 22 x 14 mm for US correlation.

**2- ABDOMEN AND PELVIS**

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I. V.C and the pelvic organs.

PATIENT: NAGLAA ATAH MOHAMMED AHMED, 10125271588, Female, 40 y

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مستشفى باهيّة أول شارع الهرم بجوار داري - الجيزة

د. النجى الأول - الشيخ زايد أمام كلية الهندسة

CT Scan

[[DOCTYPE:CT\_SCAN]]

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with lobulated outline likely fibroid.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**
- 1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

**Opinion:**

- **Stationary course since last study.**
- **No CT evidence of pulmonary or hepatic metastatic deposits.**

Reported By,  
Dr. Ali Saber, MSc  
Dr. Reham Anwar, FEBR, MD

07-Apr-2026 01:52 PM

PATIENT: NAGLAA ATAH MOHAMMED AHMED, 10125271588, Female, 40 y

Report version 2, 07-Apr-2026 01:52 PM

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د. شام عرفة اهل ساح الهم بحوار دوى .. العبرة

د. النى الاول .. الشيخ زكاه امام كلية الهندسة

CT Scan

[[DOCTYPE:CT\_SCAN]]